



Clinical Examination in Maternity and Gynaecology Unit Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Health Service	Obstetrics and Gynaecology	Midwifery and Medical

2. Purpose

At Armada Kalamunda Group (AKG), healthcare professionals have a duty to take all reasonable steps to respect the privacy, dignity, and confidentiality of patients always, especially during clinical examinations.

The specific procedure to be followed for each patient will be dependent on the urgency of the situation, the patient's age and condition of the patient and whether a parent or caregiver is present.

This guideline supports medical, nursing and midwifery practitioners in determining the indication for and appropriate conduct of vaginal examinations, with particular attention to patients in the minor and adolescent age groups.

Armada Kalamunda Group is a public health service, and as such patients will be seen based on their clinical needs without reference to the practitioner's gender, age, religion, race, or nationality.

3. Procedure:

- All patients within AKG shall be offered the presence of a chaperone during any intimate physical examinations/procedures.
- The chaperone shall: ⁵
 - Be a health employee in a clinical role, who is aware of the role of a chaperone as outlined in this procedure.
 - Respect the privacy and dignity of the patient.
- The patient's best interest (their physical, psychological health and wellbeing) is paramount and should guide all decision making.¹
- Best practice includes clear and effective communication. Health care professionals – including Doctors, Nurses, and Midwives, must take great care in explaining the procedure to the patient, particularly those in the minor and adolescent age groups, as well as to their parent or guardian, to ensure informed consent is obtained.
- Examinations should be conducted to minimise discomfort and distress.
- The patient's dignity and privacy shall be maintained throughout the examination regardless of the presence of others.

- When examining a woman, health professionals must ensure that a chaperone is available upon request for any physical examination, regardless of the health practitioner's gender. The chaperone's name should be documented in the patient's Digital Medical Record (DMR) entry with the decision as to why a vaginal examination is required.
- Doctors should discuss with the patient the benefits of having a chaperone present during the examination and offer the option to have a support person of their choice.
- A patient has the right to decline the presence of a chaperone, and the health professional also has the right to decline performing the vaginal examination if they consider it inappropriate to proceed without a chaperone.
- If a patient with decision-making capacity declines a part of or the whole examination, it must not be performed. The patient's decision to decline should be documented in their DMR.
- The practitioner undertaking the examination must ensure there is valid consent (oral or written) from the patient and in the case of minor age group, their parent or guardian prior to conducting a vaginal examination. Valid consent must be voluntary, informed, and based on the capacity of the patient to consent. The consent process should be clearly documented in the patient's DMR.
- When parents / guardians have provided consent on behalf of a minor, health professional must still explain the procedure to the young person and proceed only with her consent.
- Special considerations shall be given to obtaining consent from patients who are: -
 - Person with intellectual disability
 - Person experiencing mental illness.
 - Person with a physical disability.
 - Person affected by drugs or alcohol.
 - Person from a non-English speaking background
 - Person who is injured, in pain or in shock.
 - Person who is sleep deprived.
 - Person unable to give valid consent.

Except in a medical emergency, a vaginal examination should not proceed in the absence of valid consent.

Digital or instrumental vaginal examination is rarely indicated in minors. In cases involving allegations of sexual abuse, vaginal bleeding, abnormal discharge, or suspected genital malformations, a visual inspection of the vaginal vestibule and/or ultrasound may be appropriate. If these non-invasive methods do not provide sufficient diagnostic information and further examination is deemed medically necessary, an examination under anaesthesia—including vaginotomy—may be indicated.

An exception to the general guideline regarding vaginal examination in minors applies if the young person is pregnant.

For individuals in the minor age group, digital or instrumental vaginal examinations should only be performed with informed consent from the young person and additional consent from their parent or legal guardian.

If the young person states that she is not sexually active, such examinations are rarely warranted.

The patient must be informed in advance of the presence of medical students and has the right to decline their attendance during any examination.

3.1 Indications for speculum examination

Any gynaecological complaints including but not limited to:

- Pelvic organ prolapse.
- Symptomatic Pelvic pain
- Cervical Screening Test
- Endo-cervical swab for investigation of possible infection
- Endo-cervical swab for forensic investigation
- Abnormal vaginal bleeding
- Assessment for possible intra vaginal foreign body
- Assessment of developmental abnormality (rarely)
- Pregnancy related examinations

If the midwife or GP Obstetrician is unable to visualise the cervix, the Specialist Obstetrician/ Gynaecologist must be contacted to complete the speculum/ vaginal examination.

A result of the examination is to be documented in the patient's DMR.

Measures to minimising discomfort during pelvic examination.

- Provide explanations tailored to the age of the woman and the level of comprehension.
- Where possible, use anatomical models, pictures, and pamphlets to provide information.
- Discuss the use of any swabs or components (e.g. speculum) that will be used. Show any equipment to be used.
- Encourage the patient to empty their bladder prior to the examination.
- Conduct the examination in a calm environment and ensure privacy.
- Ensure that the equipment used is appropriate for the size and age of the patient.
- Ensure a chaperone is present and the patient is comfortable with the person present. Family members or friends of an adult patient are not appropriate to function as a chaperone. A family member may be present during the clinical examination or procedure if it is the expressed desire of the patient but should not serve as a chaperone for the purposes of this procedure.
- Encourage the patient especially minor and adolescent age group to provide feedback to the examiner if they are not comfortable, either physically or emotionally.

4. Roles and responsibilities

All staff involved in care of patient.

Role	Responsibility
Chaperone	A chaperone has the right to stop a sensitive procedure, examination, or care if they feel that the health professional's behaviour is inappropriate or unacceptable. A chaperone who witnesses inappropriate or unacceptable behaviour on the part of the health professional will immediately report this to their manager or another senior manager, even if they did not stop the procedure while it was ongoing.

Role	Responsibility
Health Professional	It is the responsibility of the health professional to ensure that accurate records are kept of the clinical contact, which also includes records regarding the use or refusal of a chaperone. It is the responsibility of the health professional to be sensitive to differing expectations regarding race, culture, ethnicity, age, gender, and sexual orientation.

5. Legislative context

The following documents are required to give effect to this procedure:

- Health Department of Western Australia (HDWA) [Consent to Treatment Policy](#)
- East Metropolitan Health Service (EMHS) [Consent Policy EMHS: 5](#)

6. Supporting information

The following documents support the implementation of this procedure:

- [Interpreter and Translator Guideline](#)

7. Compliance, monitoring, and evaluation

Compliance against this procedure will be evaluated with Chaperone Policy(health.wa.gov.au).
Random audits of case notes to check documentation of informed written or verbal consent

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.5: The health service organisation has processes to identify:
 - a. The capacity of a patient to make decisions about their own care.
 - b. A substitute decision-maker if a patient does not have the capacity to make decisions for themselves.
- Standard 2 – Action 2.8: The health service organisation uses communication mechanisms that are tailored to the diversity of the consumers who use its services and, where relevant, the diversity of the local community
- Standard 2 – Action 2.10: The health service organisation supports clinicians to communicate with patients, carers, families and consumers about health and health care so that:
 - a. Information is provided in a way that meets the needs of patients, carers, families, and consumers.
 - b. Information provided is easy to understand and use.
 - c. The clinical needs of patients are addressed while they are in the health service organisation.
 - d. Information needs for ongoing care are provided on discharge.
- Standard 5 – Action 5.3: Clinicians use organisational processes from the Partnering with Consumers Standard when providing comprehensive care to:
 - a. Actively involve patients in their own care.
 - b. Meet the patient's information needs.
 - c. Share decision-making

- Standard 6 – Action 6.5: The health service organisation:
 - a. Defines approved identifiers for patients according to best practice guidelines.
 - b. Requires at least three approved identifiers on registration and admission; when care, medication, therapy, and other services are provided; and when clinical handover, transfer or discharge documentation is generated.

9. References

1. Genital Examination in Girls and Young Women: A Clinical Practice Guideline. The Royal Australasian College of Physician (RACP). August 2009
2. Royal Australian College of General practitioners: Guidelines for Gynaecological examinations and procedures: 2015
3. Royal Australian and New Zealand College of Obstetricians and Gynaecologists. Guidelines for Gynaecological Examinations and Procedures. College Statement C- Gyn 30, 2016.
4. AMA Position Statement: Patient Examination Guidelines 2012
5. WACHS [Chaperone Policy ED-CO-17-23053](#)

10. Glossary

The following definitions are relevant to this policy.

Term	Definition
Chaperone	A chaperone should be an impartial observer to the assessment or examination, preferably a clinical member of staff, whilst acting as an advocate for the patient and preserving patient dignity. ⁶
Health Professional	A HP refers to any WACHS employed clinician governed by the Australian Health Practitioner Regulation Agency or professional body including medical, nursing, midwifery or allied health ⁵

11. Policy owner (relating to these procedures)

Head of Department Obstetrics and Gynaecology

Enquiries relating to this procedure may be directed to:

Title: Head of Department
 Department: Obstetrics and Gynaecology
 Email: Sangeeta.Mallabhat@health.wa.gov.au

12. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V5	16 10 2025	16 10 2028	Scheduled review; Minor grammatical updates.

13. Authorisation

This procedure has been authorised and issued by:

Authorised by	Dr Tania Strickland - Director Clinical Services.
Authorisation date	16 10 2025
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